

Chapter 4

Patient Rights and Education (PRE)

Intent of the chapter

The organisation defines, protects and promotes the patient and family's rights and responsibilities. The staff is aware of these rights and is trained to protect them. Patients are informed of their rights and educated about their responsibilities at the time of entering the organisation.

The expected costs of treatment and care are explained clearly to the patient and / or family.

The organisation encourages patient engagement to enhance clinical outcomes, safety and quality.

Patients are educated about the mechanisms available for addressing grievances.

Informed consent is obtained from the patient or family for specified procedures / care. The key components of information shall include risks, benefits and alternatives.

Patients and families have a right to get information and education about their healthcare needs in a language and manner that is understood by them.

The organisation has a mechanism to capture the patient experience including patient reported experience measures. (PREM).

The organisation develops effective patient-centred communication.

SUMMARY OF STANDARDS

PRE.1.	The organisation protects and promotes patient and family rights and informs them about their responsibilities during care.
PRE.2.	Patient and family rights support individual beliefs, values and involve the patient and family in decision-making processes.
PRE.3.	The patient and / or family members are educated to make informed decisions and are involved in the care planning and delivery process.
PRE.4.	Informed consent is obtained from the patient or family about their care.
PRE.5.	Patient and families have a right to information and education about their healthcare needs.
PRE.6.	Patients and families have a right to information on expected costs.
PRE.7.	The organisation has a mechanism to capture patient's feedback and to redress complaints.
PRE.8.	The organisation has a system for effective communication with patients and / or families.

*This implies that the objective element requires documentation.

Summary of Objective Elements

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Objective Element	PRE.1.	PRE.2.	PRE.3.	PRE.4.	PRE.5.	PRE.6.	PRE.7.	PRE.8.
a	Commitment	Commitment	CORE	CORE	CORE	CORE	Commitment	Commitment
b	Achievement	Commitment	Achievement	Commitment	Commitment	Commitment	Achievement	Commitment
c	CORE	Commitment	Commitment	CORE	Commitment	Commitment	CORE	Commitment
d	CORE	CORE	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment
e	CORE	Commitment	Achievement	CORE	Commitment		Commitment	Achievement
f		Commitment			Commitment		Commitment	
g		CORE			Commitment			
h		Commitment			Commitment			
i		Commitment			Achievement			
j		Commitment			Excellence			
k		Commitment						
l		Achievement						

Standards and Objective Elements

Standard

PRE.1.

The organisation protects and promotes patient and family rights and informs them about their responsibilities during care.

Objective Elements

- Commitment a. Patient and family rights and responsibilities are documented, displayed, and they are made aware of the same. ***

Interpretation: Organisation shall document the patient's rights and inform them of their responsibilities. These shall be documented in consonance with the Charter of Patients' Rights laid down by the statutory body. The rights and responsibilities of the patients shall be displayed in the hospital where it is prominently visible to patients, families and visitors. Pamphlets could be used to make them aware. Information, education and communication material shall at least be bilingual.

- Achievement b. Patient and family rights and responsibilities are actively promoted. ***

Interpretation: Organisation shall take steps to actively promote patient and family rights and responsibilities. For in-patients, the organisation shall counsel the patient and/or family on their rights and responsibilities. Counselling is done in a format and language that they can understand. For the out-patients, educational material shall be easily accessible and prominently displayed.

CORE

- c. The organisation protects patient and family rights.**

Interpretation: The first measure to protect patient and family rights is making staff aware of their responsibility in protecting and respecting patient and family rights. The staff shall conduct themselves in a manner wherein their actions convey a positive attitude towards the protection of patient and family rights.

CORE

- d. **The organisation has a mechanism to report a violation of patient and family rights.**

Interpretation: The organisation may develop a list of such instances which could be considered as infringements of patients' and families' rights and train the staff accordingly. For example, compromising the privacy, breaching confidentiality, disrespect to the religious and cultural needs, not providing medical records within the stipulated time etc. Violation of patient and family rights is reported through an incident reporting form. It shall provide details of how the right was violated and where applicable by whom. Also, there shall be a mechanism for the patient and/or family to report a violation of their rights. The patient feedback form (by incorporating patient rights worded appropriately) could be used as a tool to capture violation of patient rights.

CORE

- e. **Violation of patient and family rights are monitored, analysed, and corrective / preventive action taken by the top leadership of the organisation.**

Interpretation: Whenever patients' rights have been infringed upon, management shall document the said violations, investigate and maintain the record of the incident and its outcomes. Corrective and/or preventive actions shall be taken in a time frame defined by the organisation.

Standard

PRE.2.

Patient and family rights support individual beliefs, values and involve the patient and family in decision-making processes.

Objective Elements

- Commitment** a. **Patients and family rights include respecting values and beliefs, any special preferences, cultural needs, and responding to requests for spiritual needs.**

Interpretation: This could include how they wish to be addressed, dietary preferences and worship requirements. This may also include any specific requirement following death.

Commitment b. Patient and family rights include respect for personal dignity and privacy during examination, procedures and treatment.

Interpretation: During all stages of patient care, be it in the examination or carrying out a procedure, hospital staff shall ensure that the patient's privacy and dignity are maintained. The organisation shall develop the necessary guidelines for the same. During procedures, the organisation shall ensure that the patient is exposed just before the actual procedure. With regards to photographs/recording procedures, the organisation shall ensure that explicit informed consent is taken and that the patient's identity is not revealed.

Commitment c. Patient and family rights include protection from neglect or abuse.

Interpretation: Examples of this include falling from the bed/trolley due to negligence, assault, repeated internal examinations (unwarranted), manhandling, etc. Special precautions shall be taken, especially concerning vulnerable patients, for example elderly, neonates, physically and mentally challenged patients, comatose patients, patients under anaesthesia etc.

CORE d. Patient and family rights include treating patient information as confidential.

Interpretation: The organisation and the treating team shall take effective measures to maintain the confidentiality of all patient-related information. Staff shall avoid having patient-related discussions in public places. Statutory requirements regarding privileged communication shall be followed at all times (refer the glossary for a definition of privileged communication). Confidential information, including HIV status, shall not be revealed without the patient's permission. It shall not be explicitly written/pasted on the cover of the medical record, nor shall it be displayed in a manner that is easily understandable by the public at large.

Commitment e. Patient and family rights include the refusal of treatment.

Interpretation: The treating doctor shall discuss all the available options and allow the patient to make an informed choice. In case of refusal, the treating doctor shall explain the consequences of the refusal of treatment and document the same. After explanation of consequences, if the patient still refuses treatment, the same must be respected.

Commitment f. Patient and family rights include a right to seek an additional opinion regarding clinical care.

Interpretation: There is a mechanism for patient and family to seek a second opinion if they wish, from within or outside the organisation. The organisation shall respect the decision of the patient and family and facilitate access to all relevant information or clinical evaluation. Request for additional information on a particular physician in terms of qualifications and experience may be provided.

CORE g. Patient and family rights include informed consent before the transfusion of blood and blood components, anaesthesia, surgery, initiation of any research protocol and any other invasive / high-risk procedures / treatment.

Interpretation: Informed consent shall be obtained by treating doctor or a doctor member of the treating team.

Refer to PRE.4.a.

Commitment h. Patient and family rights include a right to complain and information on how to voice a complaint.

Interpretation: The displayed patient rights shall include the right to make a complaint and also mention the methodology to voice the same. Complaint mechanism must be accessible, and redressal of complaint must be fair and transparent.

Refer to PRE.7.c.

Commitment i. Patient and family rights include information on the expected cost of the treatment.

Interpretation: Patients and families are explained about the expected costs of treatment in a transparent manner. This includes consultations, procedures and investigations. It may involve giving written estimates or making the concerned tariff available.

Refer to PRE.6. a.

Commitment j. Patient and family rights include access to their clinical records.

Interpretation: The organisation shall ensure that every patient has access to his/her record. This shall be in consonance with the Code of Medical Ethics laid down by the Medical Council of India and statutory requirements.

Refer to IMS.4.h.

Commitment k. Patient and family rights include information on the name of the treating doctor, care plan, progress and information on their health care needs.

Interpretation: Information on the name of the treating doctor, care plan, the progress of the patient and the healthcare needs are discussed with patient and family.

Achievement l. Patient rights include determining what information regarding their care would be provided to self and family.

Interpretation: The organisation needs to evolve a mechanism to provide sensitive and/or confidential information to the patient and the next of kin if desired by the patient. In the case of minors, it will be provided to at least one of the parents / guardian.

Standard

PRE.3.

The patient and / or family members are educated to make informed decisions and are involved in the care planning and delivery process

Objective Elements

CORE

a. The patient and / or family members are explained about the proposed care (including the risks, benefits, alternatives), expected results and possible complications.

Interpretation: The proposed care, including referral to internal and/or external services, is discussed by the attending doctor with the patient and/or family members. This shall be done in a language that the patient/attendant can

understand. The above information could be documented and signed by the doctor concerned.

The patients and/or family members are explained in detail by the treating physician or his/her team about the expected outcomes of such treatment at periodic intervals.

Possible complications of the treatment, if any, are clearly communicated to the patient and/or family members.

Achievement b. The care plan is prepared and modified in consultation with the patient and / or family members.

Interpretation: During the preparation of the care plan, the patient and/or family members are explained about the various treatment options, risks and benefits. The care plan, where possible, incorporates patient and/or family concerns and requests. The religious, cultural and spiritual views of the patient and/or family shall be considered during the process of care delivery. Incorporating patient and/or family requests shall be limited by the statutory requirements. The organisation could develop a structured mechanism to implement and capture the same.

Commitment c. The patient and/or family members are informed about the results of diagnostic tests and the diagnosis.

Interpretation: The results of all diagnostic tests are explained at least in broad terms to patient and family members and their implication on progress and treatment.

Commitment d. The patient and/or family members are explained about any change in the patient's condition in a timely manner.

Interpretation: The counselling includes improvement, deterioration or occurrence of complications. Withholding of resuscitation requests from relatives and family could be discussed within ethical and legal parameters.

- Achievement e. The patient and/or family members are provided multi-disciplinary counselling when appropriate.**

Interpretation: The organisation shall identify situations where multidisciplinary counselling is required. This could include counselling of family members of critically ill patients, potential organ donors and/or family, long-stay patients.

Members of the multidisciplinary team involved in providing care like doctors from different specialities, nurses and physiotherapists shall be a part of the counselling. The counselling could be led by clinician in-charge of the patient.

Refer to COP.1.e.

Standard

PRE.4.

Informed consent is obtained from the patient or family about their care.

Objective Elements

CORE

- a. The organisation obtains informed consent from the patient or family for situations where informed consent is required. ***

Interpretation: A list of procedures shall be made for which informed consent is required. This shall be prepared to keep in mind the requirements of this standard and statutory requirement. For example, some statutory requirements are MTP Act, PC-PNDT Act and The Transplantation of Human Organs Act. The policy for HIV testing shall follow HIV and AIDS (prevention and control) act 2017 and the national policy on HIV testing laid down by National AIDS Control Organisation (NACO). The organisation shall have written guidance explaining the various steps involved in the informed consent process and the person responsible. The staff are made aware of the same.

- Commitment b. Informed consent process adheres to statutory norms.**

Interpretation: This includes (but is not limited to):

1. Taking consent before the procedure;
2. At least one witness signing the consent form.

The witness shall be a person who was present for the entire duration of the communication between the doctor and the patient.

In case the patient has to undergo a procedure repeatedly for a long time (for example dialysis), informed consent is taken at the first instance. Such consent shall have a defined validity period but not more than six months. The patient endorses the consent at each repeat treatment. However, if there is a change in the treatment modality or an addition of another modality, then fresh consent shall be obtained.

CORE

- c. Informed consent includes information regarding the procedure; its risks, benefits, alternatives and as to who will perform the procedure in a language that they can understand.**

Interpretation: The consent shall have the name of the doctor performing the procedure. In case a procedure requires more than one doctor from different specialities, then the same will have to be explained to the patient and consent shall include the name of the principal surgeon from each speciality who is performing the procedure. Each doctor will have to explain his role and address all aspects required for informed consent. For example, if the surgery involves the requirement of a Neurosurgeon, ENT surgeon and an Ophthalmologist, the consent shall reflect the same. It shall have the names of the principal surgeons of the three specialities. It is the responsibility of each of the surgeons/team to explain their role and the benefits/risks and alternatives of the procedures they are performing on the patient.

If it is a “doctor under training” the same shall be specified. However, the name of the qualified doctor supervising the procedure shall also be mentioned.

The consent form shall at a minimum be bilingual. When consent is taken in a language other than what the patient understands, there shall be clear documentation detailing the language in which the patient has been counselled and if any interpreter has been used.

It shall have the risks, benefits and alternatives of the procedure as a part of the documentation. The focus is on informed consent as a process of effective communication between a doctor and patient and not a signature on a form.

- Commitment d. The organisation describes who can give consent when a patient is incapable of independent decision making and implements the same. ***

Interpretation: The consent shall be taken from the patient in all cases when the patient is capable of giving consent and above the legal age for giving consent. No one can consent on behalf of a competent adult. The organisation

shall take into consideration the statutory norms when the patient is incapable of independent decision making. This would include next of kin/legal guardian. The order of preference of next of kin/legal guardian is spouse, son/daughter/parents/brothers/sister. For life-threatening situations when a patient is incapable and next of kin is not available, in the interest of the patient, the treating doctor and another clinician can decide to safeguard the patients' life.

CORE

e. Informed consent is taken by the person performing the procedure.

Interpretation: The person performing shall be responsible for the entire consent process, including providing explanation and taking the signature. For example, it is not acceptable if the person performing the procedure only explains, and the written consent is taken by the nurse.

A doctor member of the team can take consent on behalf of the person performing the procedure.

Refer to COP.7.e., COP.12.b., COP.13.d. and COP.14.c.

Standard

PRE.5.

Patient and families have a right to information and education about their healthcare needs.

Objective Elements

CORE

a. Patient and / or family are educated in a language and format that they can understand.

Interpretation: During the patient's treatment, patient and/or family are screened (informally) for their understanding abilities and language requirements. The patients and/or family members are educated through counselling/use of printed material/audio-visual aids etc.

- Commitment b. Patient and / or family are educated about the safe and effective use of medication and the potential side effects of the medication, when appropriate.**

Interpretation: The organisation shall make a list of such drugs and accordingly educate, for example digoxin. This could also include education regarding the importance of taking a drug at a specific time.

- Commitment c. Patient and / or family are educated about food-drug interaction.**

Interpretation: The organisation shall make a list of such drugs, and accordingly, patient and family shall be educated about their diet during medication, for example no alcohol when taking metronidazole.

- Commitment d. Patient and / or family are educated about diet and nutrition.**

Interpretation: The education could include the relationships between various foods or supplements and specific health conditions. It shall also incorporate general recommendations for following a healthy diet.

- Commitment e. Patient and / or family are educated about immunisations.**

Interpretation: In adults, it could be for influenza, Streptococcus pneumonia, typhoid, hepatitis B, Neisseria meningitides, etc.

Paediatric immunization shall follow universal immunisation program.

- Commitment f. Patient and /or family are educated on various pain management techniques, when appropriate.**

Interpretation: The education on pain management techniques could be done only for patients who are likely to have long-term pain because of the underlying condition not being treatable. This shall be done within the framework of their personal, cultural and religious beliefs.

- Commitment g. Patient and/or family are educated about their specific disease process, complications and prevention strategies.**

Interpretation: The education shall include information on lifestyle modifications (stress management, physical exercise, cessation of smoking and substance abuse), diet changes and immunisations where appropriate. The education could also be done through patient education booklets/videos/leaflets, etc.

- Commitment h. Patient and/or family are educated about preventing healthcare associated infections.**

Interpretation: For example, hand washing and avoiding overcrowding near the patient.

- Achievement i. The patients and/or family members' special educational needs are identified and addressed.**

Interpretation: During the patient's treatment, his/her special educational needs are identified. The patients and/or family members are educated through counselling, use of printed material, audio-visual aids etc. For example Attention deficit hyperactivity disorder (ADHD), needs Autism support, physical disabilities and speech language and communication needs, Social and emotional health needs.

Excellence

- j. **The organisation has a mechanism to promote patient engagement to enhance clinical outcomes, safety and quality.**

Interpretation: Patient care engagement could involve disease-based patient support groups, involvement of patient in patient safety and quality improvement.

The organisation shall create an enabling environment for partnership between patient, families, communities and health care providers.

Patients shall be encouraged to become actively involved in reporting safety incidents, near misses and safety concerns. Formation of patient advisory council in hospital is recommended as part of patient engagement.

The organisation has to recognise that patients are part of social groups, families and communities and that these broader networks can be a positive force for change.

The organisation has to designate and support patient safety champions or advocates, where appropriate, to help facilitate patient engagement;

For 'Patient engagement' refer glossary

Standard

PRE.6.

Patients and families have a right to information on expected costs.

Objective Elements

CORE

- a. **The patient and / or family members are made aware of the pricing policy in different settings (out-patient, emergency, ICU and in-patient).**

Interpretation: The key components of pricing, namely consultation charges, bed charges, nursing charges, security deposit, are available to patients near registration and/or admission desk. This shall be based on the billing policy, which defines the charges to be levied for various healthcare activities in a given setting.

Commitment

- b. **The relevant tariff list is available to patients.**

Interpretation: The organisation shall ensure that there is an updated tariff list and that the relevant tariff is available for review to patients when required. The organisation shall charge as per the tariff list. Any additional charge shall also be enumerated in the tariff and the same communicated to the patients. The tariff rates shall be uniform (in a given setting) and transparent.

Commitment c. The patient and/or family members are explained about the expected costs.

Interpretation: Patients shall be given an estimate of the expenses on account of the treatment, preferably in a written form. This estimate shall be prepared based on the treatment plan. It could be prepared by the OPD/Registration/Admission staff in consultation with the treating doctor. The limitations of the estimate if any (for example emergency admissions) could also be discussed with the patient.

Commitment d. Patient and/or family are informed about the financial implications when there is a change in the care plan.

Interpretation: Change in the care plan entails that the patient and/or family are informed of the financial implications. Example of change in care plan include patients are shifted from one setting to another, typically to and from ICUs, the shift from medical to surgical management, need to conduct further expensive investigations etc.

Standard

PRE.7.

The organisation has a mechanism to capture patient's feedback, experience and to redress complaints.

Objective Elements

Commitment a. The organisation has a mechanism to capture feedback from patients, which includes patient satisfaction.

Interpretation: The feedback could be captured either physically or electronically. It is preferable that separate data is obtained from out-patients and in-patients.

Achievement b. The organisation has a mechanism to capture the patient experience.

Interpretation: Patient experience goes beyond patient satisfaction and making the patient happy. In addition to collecting patient feedback, the organisation shall also capture patient experience which may include parameters such as communication with doctors and nurses, pain management, hospital environment (cleanliness and quietness), the responsiveness of hospital staff,

discharge information, communication about medications and overall rating of the hospitals.

The organisation has a mechanism to capture patient reported experience measures. (PREMs). Patient-reported experience measures (PREMs) are a critically important tool to ensure that the patient safety policies and protocols are implemented.

CORE

c. The organisation redress patient complaints as per the defined mechanism. *

Interpretation: The written guidance shall incorporate the mechanism for lodging complaints (including verbal or telephonic complaints), method of compiling them, analysing complaints including the time frame, the person(s) responsible and documenting the action taken. It is for the organisation to decide if it wants to give credence to anonymous complaints.

Patient complaints include those against healthcare workers.

Commitment

d. Patient and / or family members are made aware of the procedure for giving feedback and / or lodging complaints.

Interpretation: The awareness shall be either by display or providing written information. The organisation must create an environment of trust wherein the patients would be comfortable to air their views.

Commitment

e. Feedback and complaints are reviewed and / or analysed within a defined time frame.

Interpretation: The entire process shall be documented. Where appropriate, the patient and/or family could be involved in the discussions and also informed regarding the outcome.

- Commitment f. Corrective and / or preventive action(s) are taken based on the analysis where appropriate.**

Interpretation: The analysis identifies opportunities for improvement and the same are carried out.

Standard

PRE.8.

The organisation has a system for effective communication with patients and/or families.

Objective Elements

- Commitment a. Communication with the patients and / or families is done effectively. ***

Interpretation: Communication is considered to be effective if it serves the purpose. The principles of effective communication are complied. For example, the seven C's namely clear, correct, complete, concrete, concise, considerate and courteous.

The organisation has plans to identify and overcome potential communication barriers. For example, the language barrier could be overcome by having interpreters.

The organisation could adopt any model of effective communication.

Refer to the glossary for "effective communication".

- Commitment b. The organisation shall identify special situations where enhanced communication with patients and / or families would be required. ***

Interpretation: Some of these situations could include communication during challenging situations like breaking bad news, handling adverse events, handling an aggressive patient/family, talking to a family of a patient who has expired, counselling for a complicated intervention etc.

Refer to Glossary for "enhanced communication".

Commitment c. Enhanced communication with the patients and/or families is done effectively. *

Interpretation: For each identified special situation, the organisation shall detail the nature of the enhanced communication that may be required. For example, a model for delivering bad news is SPIKES (S stands for setting, P for perception, I for invitation or information, K for knowledge, E for empathy, and S for summarize or strategize.)

Commitment d. The organisation ensures that there is no unacceptable communication.

Interpretation: The organisation shall not allow unacceptable communication. For example, abusing patients, hurting the religious or cultural sentiments, communicating with disrespect, etc.

Achievement e. The organisation has a system to monitor and review the implementation of effective communication.

Interpretation: This could be done through feedback/ complaint from patients and other stakeholders.

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